CenterWell Home Health

PREPARED FOR
CWHH CLINICAL QUALITY TEAM

Clinical Quality Intelligence

Which patient needs care today, before it's too late?

Weekly Patient Conference Summary

High-Risk & Rising-Risk Patients | Week of 08/17/2026

CQI is a framework for identifying risk, care gaps and opportunities to improve patient outcomes. It is **proactive**: it shows which patient needs care this week, why, and what to do **while there is still time to change the outcome**.

Patient

NEARING DISCHARGE DECISION: 14 DAYS

Patient ID	**9000345**	60-day risk	**Moderate 0.20**
Episode	**7000588**	Status	**Recertified, day 45 of 60**
Branch	**Branch 0518**	CQI run	**08/15/2026**

Produced by **CW Data Science & Applied AI - HH Team** Model **cwhh-tif-60day-v1**

What CQI is: page 2. How to use this booklet: page 3.

CLINICAL QUALITY INTELLIGENCE

What is Clinical Quality Intelligence?

Purpose, how it works and what's in this booklet

CQI answers: Which patient needs care **today**, before it's too **late**? What **immediate actions** could a clinical manager take to **prevent** hospitalizations?

Most quality measures are **retrospective**: they report what happened after the episode ends. CQI is **proactive**: every week it connects risk, the care ordered and delivered, and the patient's trajectory, so the team can act **while the patient is still in care**.

Purpose: **Weekly Patient Conference Preparation | Built for the CWHH Clinical Quality team**

What CQI does

Compares the care each high-risk patient **needs** with the care they **received**, so clinicians can close gaps early.

What it reads

Visits, goals, vitals, OASIS, wounds, medications, events and notes. Co-created with CWHH Clinical Quality.

How AI is used

Predictive ML risk model on LighthouseAI; **rules** for care gaps; **LLMs** summarize, checked by LLM judges.

What's in this booklet

Care gaps
Missed, delayed or unscheduled visits
Pages 21-22 >

Risk drivers
Who is highest risk this week, and why
Page 10 >

Clinical picture
Diagnoses, wounds and OASIS
Pages 18-20 >

Vitals trajectory
Trend, urgency and action window
Page 11 >

Patient goals
Met, not met, therapy goals
Page 12 >

Medication adherence
High-risk meds, understanding, teaching
Page 13 >

Care plan gaps
Interventions, assessments, visit goals to add
Pages 14, 24 >

Discharge & network
Recommendation, PCP, CW PCO, CW Rx
Pages 9, 15 >

Where CQI is going

Clinicians stay in charge of every decision.

TODAY | PROTOTYPE

A weekly booklet and Power BI report built from operational data, reviewed by the clinical team in the patient conference. Clinician feedback shapes what comes next.

IN PRODUCTION | THE VISION

An **AI-assisted agent** that scans operational data **24/7 on clinicians' behalf**, surfaces care gaps and opportunities to improve outcomes **as they emerge**, and brings the right patient to the right clinician while there is still time to act.

Inside this booklet

How to use it in the weekly patient conference, and where to find each section

How to use this booklet in conference

1. **Start with At a glance (page 4), then the one-page summary (page 5).**
   Why the patient is on the list, four status tiles and the recommended actions.
2. **Review the CQI analysis (pages 6-15).**
   What CQI found, concluded and recommends: CQI summaries, risk, vitals, goals, medications, care plan gaps.
3. **Check it against the evidence (pages 16-25).**
   The operational records behind each finding. Each page ends with a feedback tip.
4. **Give your input on screen (pages 26-28).**
   Rate each finding, record the conference decision, save and return.

Contents

CQI analysis pages show what CQI worked out: predicted risk, rule-based checks and AI-written summaries. **Evidence** pages show the operational records behind them, so you can check CQI.

Start here

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CQI analysis

Section starts p. 6

What CQI found, concluded and recommends

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CQI summaries: care, missed care, data gaps	8
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Evidence

Section starts p. 16

The records behind the findings, and how CQI used them

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OASIS	20
Visits: ordered, delivered, scheduled	21
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How CQI produced this	25

Your input

Section starts p. 26

Fillable on screen: feedback, decision, notes

Clinician feedback	27
Conference decision and notes	28

Colour key

Act now / urgent, missed, or a data gap to resolve · In progress / this week · Monitor · On track / met

At a glance

From the patient's stated goal to what needs attention now

From stated goal to what needs attention now

Patient goal	*“Walk to church by myself again.”*
Risk	Moderate 60-day hospitalization risk (0.20); driven by ambulation needs assistance, daily pain interfering with activity, recent knee replacement.
Care delivery	3 ordered visits remain; 3 scheduled; No visit shortfall to date; Frequency status: on track.
Clinical trajectory	Pulse 76 -> 70; Systolic BP 128 -> 126; SpO2 97 -> 97. No vitals outside expected range; continue monitoring.
CQI flags	**Discharge decision due in 14 days: system suggests discharge; confirm as a team. Check medication understanding with teach-back on high-risk meds (Oxycodone) at the next SN visit.**

ACTION WINDOW	VITALS TRAJECTORY	DATA GAPS	DISCHARGE
Continue routine monitoring	Stable	2	Discharge · 14d

FEEDBACK TIP Before rating **At a glance + CQI flags** on **page 27** (fillable), check: the goal is the patient's own wording; each row matches the evidence pages; the CQI flags are the right next steps.

CWHH | WEEKLY PATIENT CONFERENCE

Clinical Quality Intelligence

Patient conference summary

Proof of concept: High-Risk & Rising-Risk Patients

cwhh-tif-60day-v1 | CQI run 08/15/2026 | Week of 08/17/2026

Patient ID	9000345
Episode	7000588
Branch	Branch 0518
Status	RECERTIFIED
Admission	New Admission
Episode day	45 of 60

60-DAY HOSPITALIZATION RISK	PERCENTILE	TREND	CARE GAPS RAISE RISK	DELAY IMPACT
Moderate 0.20	Top 67%	Rising +0.08 vs. prior run	No	Low

NEARING DISCHARGE DECISION **14 days to episode end (08/29/2026)** System: Discharge (page 9)

Prognosis: Good | Rehab potential: Good | Goals: 0 of 1 achieved | Visits left: 3 ordered, 3 scheduled

Why this patient is on today's list

Moderate and rising hospitalization risk.

IN THE PATIENT'S WORDS

"Walk to church by myself again."

Care delivery gaps — MONITOR

- 3 ordered visits remain; 3 scheduled
- No visit shortfall to date
- Frequency status: on track

Summary p.8 | Evidence p.21 >

Clinical risk drivers — MONITOR

- 60-day risk 0.20, top 67%; rising +0.08
- Top drivers: Ambulation needs assistance; Daily pain interfering wi...
- Care gaps not raising risk

Summary p.7 | Evidence p.10 >

Vitals trajectory — ON TRACK

- Pulse: -6 in 5 wks (76 to 70)
- Systolic BP: -2 in 5 wks (128 to 126)
- Last vitals day 36 (9 days ago)

Summary p.7 | Evidence p.11 >

Patient goals — ON TRACK

- 1 care-plan goals: 0 achieved, 1 in progress

Summary p.7 | Evidence p.12 >

Recommended actions for discussion

2 DATA GAPS - CHECK PAGE 8 FIRST

#	Action	Owner	When	Page
1	Discharge decision due in 14 days: system suggests discharge; confirm as a team	CM / Clinician	**This week**	9
2	Check medication understanding with teach-back on high-risk meds (Oxycodone) at the next SN visit	SN	**Next SN visit**	13
3	Progress gait training and reassess pain plan with MD.	PT	**Next visit**	12

Record the conference decision on page 28 and rate findings on page 27 (fillable) >

SECTION 1

CQI analysis

What CQI found, concluded and recommends

Pages 6-15

Everything in this section was **produced by CQI**: a predictive risk model, rule-based checks across visits, vitals, goals and medications, and AI-written summaries checked by LLM judges. This is what operational reports do not do: CQI **connects** the data, **explains** why the patient is at risk, and **recommends** what to do next.

Treat every finding as a draft for clinical review.

In this section

CQI summaries: profile, risk, vitals — 7
AI-generated, unedited

CQI summaries: care, missed care, data gaps — 8
AI-generated, unedited

Discharge or recertification — 9
System recommendation and rationale

Risk drivers and episode timeline — 10
Why the model flags this patient

Vitals trajectory and action window — 11
Trend, urgency and when to act

Goals met and not met — 12
Including therapy goals

Medication adherence — 13
High-risk meds, understanding, teaching

Care plan gaps: what to add — 14
Interventions, assessments, visit goals

Care network and referrals — 15
PCP, CW PCO, CW Rx at discharge

CQI ANALYSIS

CQI summaries: profile, risk and vitals

AI-generated text, printed unedited. Check each summary against its evidence page.

Patient 9000345 | Episode 7000588 | Branch 0518 | Day 45 of 60

Visits completed	Scheduled visits	Missed care	IP events	Vitals days	Wound patient
18	3	0	0	5	Yes

Schedule status	Risk level	Frequency status	Care gaps raise risk	Delay impact	Acuity window missed	Episode status	Discharge decision
needs change	moderate	on track	FALSE	low	FALSE	RECERTIFIED	Yes, 14 days

Patient goal (care plan)

PATIENT WILL AMBULATE 300 FT WITH CANE ON LEVEL SURFACES WITH MODIFIED INDEPENDENCE WITHIN 6 WEEKS

Patient Profile & Risk Summary

Evidence: pages 17-10

Moderate risk driven by ambulation assistance and daily pain after knee replacement; risk rose this week as pain scores increased.

Vitals Trajectory Summary

Evidence: page 11

Vitals are stable across five readings: SBP 124-130, pulse 70-78, SpO2 97%. No escalation is needed; continue routine monitoring at PT visits.

FEEDBACK TIP Before rating **Profile & Risk Summary**, **Vitals + action window** on **page 27** (fillable), check: the profile, diagnoses and living situation match the chart (page 17); the trends described match the vitals charts (page 11).

CQI ANALYSIS

CQI summaries: care delivery and data gaps

Care delivery, missed care, delay impact and data gaps, printed unedited.

Patient 9000345 | Episode 7000588 | Branch 0518 | Day 45 of 60

Care Delivery Summary

Evidence: page 21

PT met or exceeded ordered frequency in every completed week, and the SN evaluation and follow-up were completed. Schedule: three PT visits are scheduled for weeks 7-9 against three ordered, but the week 7 visit falls on DAY+46 and needs confirmation after the recertification.

Missed Care Summary

Evidence: page 22

No summary was generated for this run.

Clinical Delay Impact Explanation

Evidence: page 21

No summary was generated for this run.

Data Gaps Found

2 TO RESOLVE

Missing or conflicting data CQI noticed - confirm in the chart first.

- Recertification on DAY+60 is not reflected in the scheduled-visit data.
- Pain scores are referenced in the risk drivers but no pain assessment values are in the payload.

FEEDBACK TIP Before rating **Care Delivery Summary**, **Missed Care Summary**, **Delay Impact Explanation**, **Data Gaps Found** on **page 27** (fillable), check: visit counts match the weekly grid (page 21); missed visits and reasons match the log (page 22); each data gap is real and not already resolved in the chart.

CQI ANALYSIS

Discharge or recertification

System recommendation from this patient's data, with the rationale for each side

Patient 9000345 | Episode 7000588 | Branch 0518 | Day 45 of 60

NEARING DISCHARGE DECISION **14 days to episode end (08/29/2026)** **System: Discharge**

Prognosis: Good | Rehab potential: Good | Goals: 0 of 1 achieved | Visits left: 3 ordered, 3 scheduled

SYSTEM RECOMMENDATION

Discharge

LEANING

Most signals support discharge (5 vs. 2): Vitals stable; Wound healed; Moderate 60-day hospitalization risk (0.20). Still open: Goal not yet met (Ambulate 300 ft with cane within 6 weeks).

Supports recertification (2)

- Goal not yet met (Ambulate 300 ft with cane within 6 weeks).
- Rehab potential good with goals still progressing.

Supports discharge (5)

- Vitals stable.
- Wound healed.
- Moderate 60-day hospitalization risk (0.20).
- Function improved on OASIS: M1850 1->0, M1860 2->1, M1242 3->1.
- Prognosis good.

Check first: 2 data gap(s) to resolve before deciding.

Next steps if the team agrees

- Send the discharge summary to the patient's CW PCO team.
- Confirm a PCP follow-up within 7 days of discharge.
- Reconcile medications and teach back the discharge plan with the patient / caregiver.
- Document the outcome of the unmet goal: Ambulate 300 ft with cane within 6 weeks.
- Complete the discharge OASIS.

Rule-based recommendation for clinical review. The clinical team and physician make the discharge or recertification decision.

FEEDBACK TIP Before rating **Discharge / recert recommendation** on **page 27** (fillable), check: each signal matches the evidence pages; the recommendation fits the patient's goal and situation; the physician agrees before the plan changes.

CQI ANALYSIS

Episode timeline and risk drivers

Where the patient is in the episode, and why the model flags them

Patient 9000345 | Episode 7000588 | Branch 0518 | Day 45 of 60

Evidence for the CQI Patient Profile & Risk Summary and risk tile on page 7.

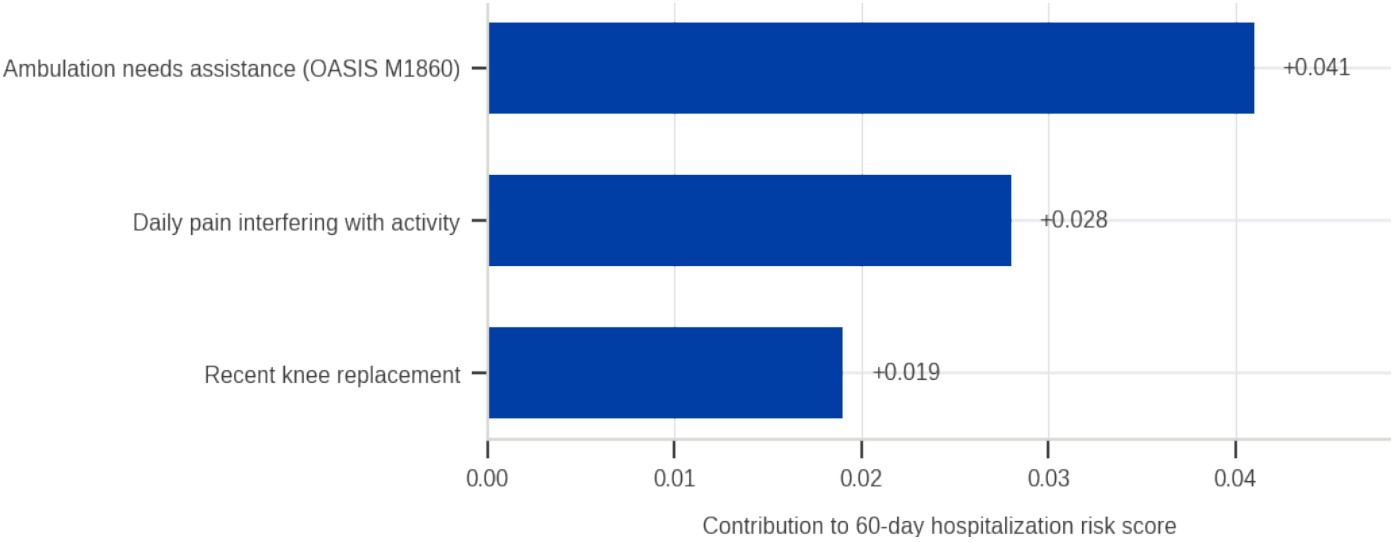
Episode timeline



NEARING DISCHARGE DECISION **14 days to episode end (08/29/2026)** **System: Discharge (page 9)**

Prognosis: Good | Rehab potential: Good | Goals: 0 of 1 achieved | Visits left: 3 ordered, 3 scheduled

Top drivers of 60-day hospitalization risk



FEEDBACK TIP Before rating **Risk tier and drivers**, **Discharge / recert recommendation** on **page 27** (fillable), check: the top drivers are clinically plausible for this patient; the prior inpatient stay and OASIS items match the chart.

CQI ANALYSIS

Vitals trajectory: action window

Weekly readings turned into a trajectory, an urgency and an action window

Patient 9000345 | Episode 7000588 | Branch 0518 | Day 45 of 60

Evidence for the CQI Vitals Trajectory Summary on page 7.

WEEKLY VITALS
5 vitals days, last DAY+36

TRAJECTORY
Stable

URGENCY
Routine

ACTION WINDOW
Continue routine monitoring

What CQI saw

Pulse 76 -> 70; Systolic BP 128 -> 126; SpO2 97 -> 97.

What CQI flags

No vitals outside expected range; continue monitoring.

VITALS DAYS	DAYS SINCE LAST READING	NEEDING REVIEW	TRAJECTORY
5	9	0	Stable

What CQI found, by measure

Measure	First	Latest	Change	CQI interpretation	Urgency
Pulse (bpm)	76	70	-6 in 5 wks		Routine
Systolic BP (mmHg)	128	126	-2 in 5 wks		Routine
SpO2 (%)	97	97	+0 in 5 wks		Routine

FEEDBACK TIP Before rating **Vitals + action window** on **page 27** (fillable), check: readings match the visit documentation (units, dates, values); the trajectory and action window are clinically appropriate; the trend charts on page 23 support what CQI saw.

CQI ANALYSIS

Patient goals

Goals met, goals not yet met, and therapy goals

Patient 9000345 | Episode 7000588 | Branch 0518 | Day 45 of 60

PATIENT-STATED GOAL (care plan wording, unchanged)

"Walk to church by myself again."

GOALS MET	NOT YET MET	DELAYED / MISSED	THERAPY GOALS (PT/OT/ST)
0	1	0	1

Goals not yet met

Goal	Disc.	CQI status	Evidence found
Ambulate 300 ft with cane within 6 weeks	PT	In progress	Ambulating 150 ft DAY+36

Goals met

Goal	Disc.	CQI status	Evidence found
None			

Therapy goals (PT / OT / ST)

Goal	Disc.	CQI status	Evidence found
Ambulate 300 ft with cane within 6 weeks	PT	In progress	Ambulating 150 ft DAY+36

FEEDBACK TIP Before rating **Goals met / not met** on **page 27** (fillable), check: goal wording and status match the care plan; 'met' goals have documented evidence; therapy goals match PT / OT / ST plans.

CQI ANALYSIS

Medication adherence

Does the patient understand, have and take their medications as prescribed?

Patient 9000345 | Episode 7000588 | Branch 0518 | Day 45 of 60

CQI summary

No summary was generated for this run.

ACTIVE MEDS	HIGH-RISK MEDS	UNDERSTANDING	ADHERENCE ISSUES	DAYS SINCE CHECK
4	1	2 of 2 visits	0	36

Flag: High-risk medications and no medication-understanding check in 36 days. Check understanding with teach-back and confirm the patient is taking each medication as prescribed.

High-risk medications

Medication	Class	Why it matters
Oxycodone 5 mg PRN	Opioid	Opioid: needs teaching and monitoring

Understanding, teaching and adherence at visits

Visit	Discipline	Understanding assessed	Teaching done	Taking as prescribed
DAY+2	**SN**	**Yes**	**Yes**	**Yes**
DAY+9	**SN**	**Yes**	**No**	**Yes**

FEEDBACK TIP Before rating **Medication adherence** on **page 27** (fillable), check: the high-risk list matches the medication profile; understanding, teaching and adherence are documented on the visits shown; adherence barriers (vision, cost, cognition, supply) are addressed in the plan.

CQI ANALYSIS

Care plan gaps: what to add

Interventions missing per goal, assessments or vitals to add, and visit goals to add

Patient 9000345 | Episode 7000588 | Branch 0518 | Day 45 of 60

Interventions missing per goal

Care-plan goal	Missing intervention	Disc.
None identified		

Assessments or vitals to add

Assessment / vital	Type	Why it is needed
None identified		

Visit goals to add to meet patient goals

Disc.	Visit goal	Supports patient goal
None identified		

Medications, supplies and equipment linked to goals

Item	Type	Supports goal	CQI alignment check
Not yet compared			

FEEDBACK TIP Before rating **Care plan gaps to add**, **Medications and supplies** on **page 27** (fillable), check: each missing item is truly missing (not just undocumented); suggested visit goals are measurable and fit the patient's goal; medications and supplies match the current med profile.

CQI ANALYSIS

Care network and discharge referrals

Primary care, CenterWell Primary Care (CW PCO) and CenterWell Pharmacy (CW Rx)

Patient 9000345 | Episode 7000588 | Branch 0518 | Day 45 of 60

NEARING DISCHARGE DECISION **14 days to episode end (08/29/2026)** **System: Discharge (page 9)**

Prognosis: Good | Rehab potential: Good | Goals: 0 of 1 achieved | Visits left: 3 ordered, 3 scheduled

Care network: PCP on record | CW PCO: Already enrolled | CW Rx: Not indicated (page 15)

Refer to CenterWell Primary Care (CW PCO)?

Already enrolled

Patient is a CW PCO patient; send the discharge summary to the CW PCO team.

Refer to CenterWell Pharmacy (CW Rx)?

Not indicated

4 active medications (below 5).

Care network

Item	Status	Detail
Primary care provider (PCP)	Yes	CenterWell Senior Primary Care - Rock Hill
Last PCP visit	26 days ago	
Enrolled in CW PCO	Yes	
Enrolled in CW Rx	No	
Plan eligible (MA)	Yes	
Nearest CW PCO clinic	3.1 mi	CenterWell Senior Primary Care - Rock Hill
CW PCO clinic within 10 miles	Yes	1 clinic(s)
CW PCO clinic within 20 miles	Yes	2 clinic(s)
Active medications	4	

How the referral indicator is set

- **CW PCO:** Refer if not enrolled, plan-eligible and a clinic is within 10 miles (stronger when there is no PCP). Consider if the nearest clinic is 10-20 miles away. Not indicated if none within 20 miles or not eligible.
- **CW Rx:** Refer if not enrolled, plan-eligible and taking 5 or more active medications. Already enrolled: share the discharge medication list.
- **Check eligibility** appears when plan eligibility is unknown. Indicators are prompts for the discharge conversation, not orders.

FEEDBACK TIP Before rating **Care network + referrals** on **page 27** (fillable), check: PCP and enrollment match the member record; the referral indicator makes sense for this patient; the patient or caregiver agrees before any referral.

SECTION 2

Evidence

The records behind the findings, and how CQI used them

Pages 16-25

These pages show the **operational records** CQI used, organized for this patient: the same data you would find in the EMR and standard reports. Use them to **confirm or challenge** the findings in CQI analysis. The last page explains **how CQI produced** those findings: methods, data sources and known limits.

Each page ends with a feedback tip that links back to the finding it supports.

In this section

Patient context and 485 profile — Demographics, living situation, profile — 17

Diagnoses and procedures — Codes, onset and exacerbation — 18

Wounds — Type, size over time, healing — 19

OASIS — Start of care vs. latest — 20

Visits: ordered, delivered, scheduled — Weekly grid by discipline — 21

Missed care and events — Missed visits, alerts, logs, inpatient — 22

Vitals trends — Charts against patient limits — 23

Assessments, interventions, adverse events — What was done at visits — 24

How CQI produced this — Methods, data sources, limits — 25

EVIDENCE

Patient context

Who the patient is: demographics, diagnoses, living situation and 485 / OASIS profile

Patient 9000345 | Episode 7000588 | Branch 0518 | Day 45 of 60

Evidence for the CQI Patient Profile & Risk Summary on page 7.

Patient context

Age band / sex	70-74 / F	**Clinical group**	MS_REHAB
Primary diagnosis	Presence of right artificial knee joint (Z96.651)	**Comorbidities**	Not found
Living situation	Not found	**Admission type**	New Admission
Acuity level / first visit	Level 3; first visit DAY+1		

Clinical profile (485 / OASIS)

Comorbidity adjustment	None	**Functional impairment**	Medium
Prognosis	Good	**Rehab potential**	Good
Functional limitations	Ambulation, pain interferes with activity	**Activities permitted**	Partial weight bearing, cane, up as tolerated
Mental status	Oriented	**Allergies**	ADHESIVE
Nutritional requirements	Regular diet		

NEARING DISCHARGE DECISION **14 days to episode end (08/29/2026)** **System: Discharge** **(page 9)**

Prognosis: Good | Rehab potential: Good | Goals: 0 of 1 achieved | Visits left: 3 ordered, 3 scheduled

FEEDBACK TIP Before rating **Profile & Risk Summary** on **page 27** (fillable), check: diagnoses, comorbidities and living situation match the chart; allergies, mental status and activities permitted are current on the 485.

EVIDENCE

Diagnoses and procedures

Primary and secondary diagnoses, onset or exacerbation, and procedures this episode

Patient 9000345 | Episode 7000588 | Branch 0518 | Day 45 of 60

CQI summary

No summary was generated for this run.

DIAGNOSES	PROCEDURES	ONSET / EXACERBATION	BODY SYSTEMS
1	1	1	1

Diagnoses

Code	Description	Body system	Clinical group	Onset / exacerbation
Z96.651	**Primary:** Presence of right artificial knee joint	Factors Influencing Health	MS_REHAB	**Onset (DAY-3)**

Procedures

Code	Description	Clinical group	Date
0SRC0J9	Replacement of right knee joint	MS_REHAB	

FEEDBACK TIP Before rating **Diagnoses, wounds, OASIS** on **page 27** (fillable), check: diagnosis codes and the primary diagnosis match the 485 / plan of care; onset vs. exacerbation is coded correctly; the procedures listed belong to this episode.

EVIDENCE

Wounds

Wound type, location, size over time and healing status

Patient 9000345 | Episode 7000588 | Branch 0518 | Day 45 of 60

CQI summary

No summary was generated for this run.

WOUNDS	PRESSURE ULCERS	HEALED	NOT MEASURED IN 7+ DAYS
1	0	1	0

Wound inventory

Location	Type	Stage	Size: first → latest	Change	Status	Count / last
KNEE, RT	Surgical Surgical Wound	-	6x0.4 cm → 0x0 cm	**-100% area**	**Healed**	3 / D+29

Wound descriptions

- **KNEE, RT:** Surgical incision - right tka

FEEDBACK TIP Before rating **Diagnoses, wounds, OASIS** on **page 27** (fillable), check: wound type, location and stage match the wound assessment; measurements and healing status are current; every open wound has a treatment order.

EVIDENCE

OASIS

Key OASIS items at start of care and the latest assessment, with changes highlighted

Patient 9000345 | Episode 7000588 | Branch 0518 | Day 45 of 60

CQI summary

No summary was generated for this run.

ITEMS CAPTURED	KEY RISK ITEMS	CHANGED SINCE SOC	ASSESSMENTS
3	3	3	2

OASIS items (key risk items first)

Item	Description	Start of care	Latest
M1850	Transferring	1 - Transfers with minimal human assistance or device	**0 - Able to independently transfer**
M1860	Ambulation / locomotion	2 - Requires use of a two-handed device (walker)	**1 - With a one-handed device (cane)**
M1242	Frequency of pain interfering with activity	3 - Daily but not constantly	**1 - Less often than daily**

Assessments: 01 - Start of care; 05 - Other follow-up. Green rows changed since start of care.

FEEDBACK TIP Before rating **Diagnoses, wounds, OASIS** on **page 27** (fillable), check: answers match the OASIS in the EMR; changed items reflect a real change in the patient; M1033 risk factors are addressed in the care plan.

EVIDENCE

Care delivery

Ordered vs. delivered vs. scheduled visits by discipline and week

Patient 9000345 | Episode 7000588 | Branch 0518 | Day 45 of 60

Evidence for the CQI Care Delivery Summary and Delay Impact on page 8.

Weekly visit frequency

Discipline	Wk 1	Wk 2	Wk 3	Wk 4	Wk 5	Wk 6	Wk 7	Wk 8	Wk 9	Total	Left / sched.
SN	1/1	1/1	-	-	-	-	-	-	-	2/2	0 / 0
PT	3/3	3/3	3/3	2/2	2/2	2/2	1 of 1	1 of 1	1 of 1	15/15	3 / 3

Weeks 1-6: delivered/ordered (green = met, plum = short). Weeks 7-9: scheduled of ordered.
Light blue rule = CQI run date. Left / sched. = ordered visits remaining vs. visits on the calendar.

FREQUENCY STATUS	SCHEDULE STATUS	ACUITY WINDOW MISSED	MISSED-CARE RECORDS
On track	Needs change	No	0

FEEDBACK TIP Before rating **Care Delivery Summary**, **Delay Impact Explanation** on **page 27** (fillable), check: ordered frequencies match the 485 / plan of care; delivered counts match the visit record; the future schedule is confirmed in the scheduling system today.

EVIDENCE

Missed care and events

Missed or outstanding visits, inpatient events, vitals alerts and client log notes

Patient 9000345 | Episode 7000588 | Branch 0518 | Day 45 of 60

Evidence for the CQI Missed Care Summary on page 8.

Missed and outstanding care

When	Discipline	Visit	Reason / evidence	Status
-		No missed or outstanding care found		

Inpatient events, vitals alerts and client log notes

When	Type	Detail	Source / follow-up
-		No events, alerts or log notes found	

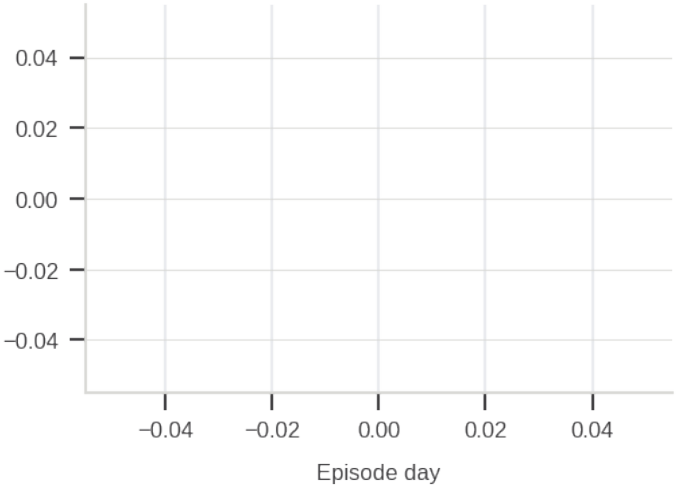
FEEDBACK TIP Before rating **Missed Care Summary** on **page 27** (fillable), check: missed visits and reasons match the visit record and client logs; alerts flagged in plum had no documented physician notification.

Vitals trends

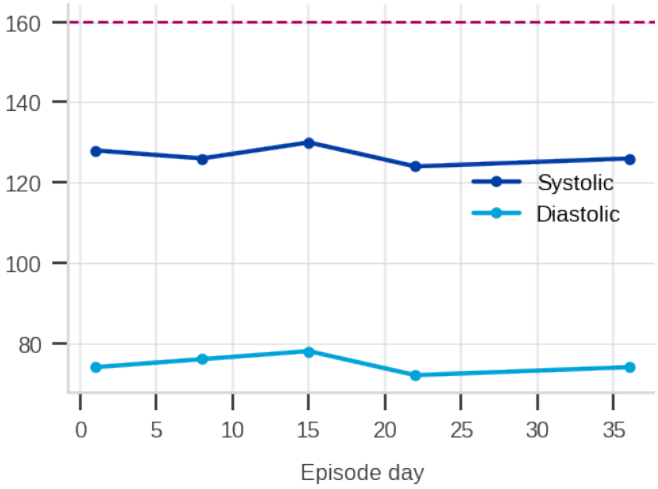
Each measure against the patient's limits, by episode day

Evidence for the CQI vitals action window on page 11.

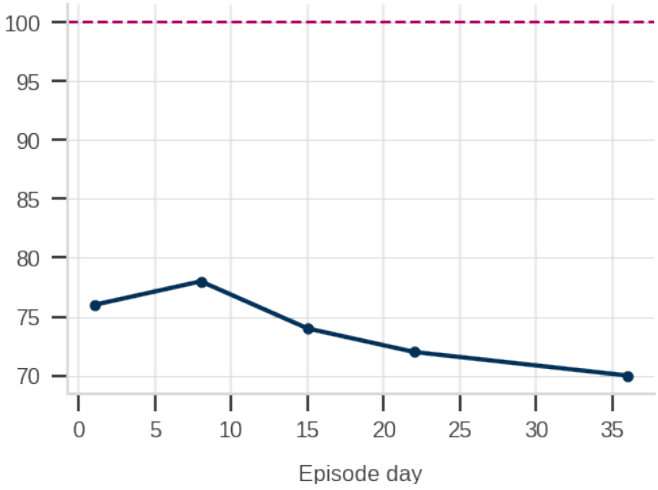
Weight (lb) - baseline dotted



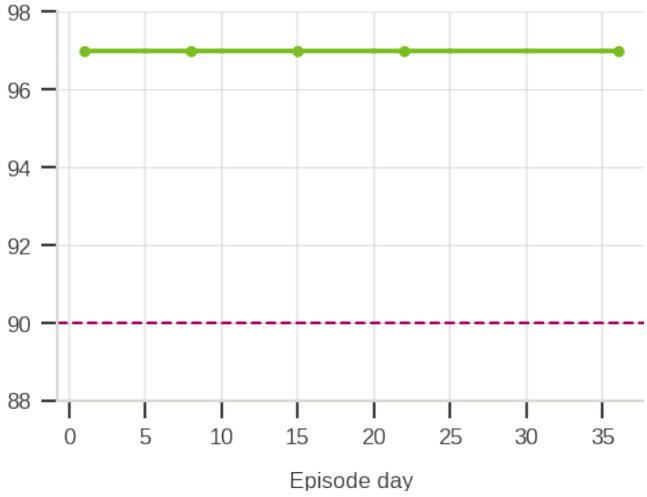
Blood pressure (mmHg) - limit SBP 160



Pulse (bpm) - limit 100



SpO2 (%) - limit 90



FEEDBACK TIP Before rating **Vitals + action window** on **page 27** (fillable), check: readings match the visit documentation (units, dates, values); limits match the patient's ordered call parameters.

EVIDENCE

Care delivered

Assessments and interventions performed, adverse events and visit narrative themes

Patient 9000345 | Episode 7000588 | Branch 0518 | Day 45 of 60

Assessments performed

No summary was generated for this run.

Assessment	Disc.	Count	Last day
Pain assessment	PT	6	DAY+36
Knee range of motion	PT	4	DAY+43

Interventions performed

No summary was generated for this run.

Intervention	Disc.	Linked care-plan goal	Count	Last day
Gait training with cane	PT	Ambulate 300 ft with cane within 6 weeks	6	DAY+38

Adverse event summary

No adverse events recorded.

Visit narrative themes

No visit narrative themes were generated for this run.

FEEDBACK TIP Before rating **Assessments + interventions**, **Adverse events + themes** on **page 27** (fillable), check: counts match the visit documentation; the summaries describe what was actually done, not what was ordered; every adverse event is listed and correctly classified.

EVIDENCE

How CQI produced this summary

Methods, data sources and definitions: what to trust and what to check

Patient 9000345 | Episode 7000588 | Branch 0518 | Day 45 of 60

From data to findings

1 Predict

Production 60-day hospitalization risk model scores high and rising-risk patients each week and explains top drivers.

2 Compare

Rules compare ordered, scheduled and delivered visits by discipline and week, timing windows and vitals limits.

3 Read

Language models read goals, care plans, notes and logs to draft the summaries on pages 7-8.

4 Check

AI drafts are checked by a panel of LLM judges and adjudicated. Clinician feedback from this pilot will guide improvements.

Data sources

Home Health operational data on the CW Lighthouse AI environment: admissions and episodes, visits, 485 plan of care and orders, vitals, assessments, interventions, OASIS, inpatient events, events, client logs, goals, notes, diagnoses and procedures, referrals, PDGM periods, supplies, medications, care-plan tasks, wounds.

Model outputs: CWHH 60-day transfer-to-inpatient risk scores and risk drivers.

Definitions

High / rising risk	High: top 25% of weekly risk scores. Rising: weekly score increasing versus prior runs.
Frequency status	on_track, minor_shortfall, behind or significantly_behind versus ordered visits to date.
Schedule status	Whether remaining ordered visits are on the calendar (e.g. no_future_schedule, needs_change).
Risk raised by care gaps	CQI judgment that missed or delayed care materially increased risk.
Delay clinical impact	Estimated clinical impact of timing delays: low, moderate or high.
Nearing discharge decision	Active episode within 14 days of the episode end date.
Urgency	Within 24 hrs, this week, or next visit - a recommendation for the team, not an order.

Known limits

- Findings reflect data available at the run date; documentation lag can look like a care gap.
- Missing goals or vitals may be a documentation gap rather than a care gap - check the chart.
- Recommendations are drafts for clinical review.

Questions or corrections

CW Data Science & Applied AI - HH Team | For more information visit go/lighthouse

SECTION 3

Your input

Fillable on screen: feedback, decision, notes

Pages 26-28

Your review makes CQI better. Rate each finding, record the conference decision and add notes **on screen**, then save and return the PDF. Answers are collected automatically.

Open in Adobe Acrobat Reader, Microsoft Edge or Chrome to fill in.

In this section

Clinician feedback 27
Rate each finding

Conference decision and notes 28
Decision, owner, follow-ups

YOUR INPUT

Clinician feedback (fillable)

Complete on screen, save and return. One form per patient; answers are collected automatically.

Patient 9000345 | Episode 7000588 | Branch 0518 | Day 45 of 60

How to complete: open this PDF in Adobe Acrobat Reader, Microsoft Edge or Chrome, click the circles and boxes, type comments, then save (Ctrl+S) and return it to the CW Data Science & Applied AI - HH Team. **Tip:** rate at least the findings you discussed. For *Partly* or *Disagree*, tick an error type and add one line saying what was wrong - that is what improves CQI.

CQI finding	Pages	Agree	Partly	Disagree	Error type D I C N	Comment (what was wrong or missing)
At a glance + CQI flags	4	☐	☐	☐	☐ ☐ ☐ ☐	
Summary page + actions	5	☐	☐	☐	☐ ☐ ☐ ☐	
Profile & Risk Summary	7, 17	☐	☐	☐	☐ ☐ ☐ ☐	
Risk tier and drivers	10	☐	☐	☐	☐ ☐ ☐ ☐	
Vitals + action window	7, 11	☐	☐	☐	☐ ☐ ☐ ☐	
Care Delivery Summary	8, 21	☐	☐	☐	☐ ☐ ☐ ☐	
Missed Care Summary	8, 22	☐	☐	☐	☐ ☐ ☐ ☐	
Delay Impact Explanation	8, 21	☐	☐	☐	☐ ☐ ☐ ☐	
Data Gaps Found	8	☐	☐	☐	☐ ☐ ☐ ☐	
Goals met / not met	12	☐	☐	☐	☐ ☐ ☐ ☐	
Medications and supplies	14	☐	☐	☐	☐ ☐ ☐ ☐	
Assessments + interventions	24	☐	☐	☐	☐ ☐ ☐ ☐	
Adverse events + themes	24	☐	☐	☐	☐ ☐ ☐ ☐	
Care plan gaps to add	14	☐	☐	☐	☐ ☐ ☐ ☐	
Diagnoses, wounds, OASIS	18-20	☐	☐	☐	☐ ☐ ☐ ☐	
Medication adherence	13	☐	☐	☐	☐ ☐ ☐ ☐	
Care network + referrals	15	☐	☐	☐	☐ ☐ ☐ ☐	
Discharge / recert recommendation	9	☐	☐	☐	☐ ☐ ☐ ☐	

Error type: D = data missing I = data incorrect C = clinical interpretation N = not relevant for this patient

Conference outcome (tick all that apply)

☐ Plan updated ☐ Visits rescheduled ☐ MD notified ☐ No change needed ☐ Refer to Data Science

How useful was this booklet for conference?

☐ 1 Not useful ☐ 2 ☐ 3 ☐ 4 ☐ 5 Very useful

What should CQI add, remove or change?

Reviewer name

Role / discipline

Date

YOUR INPUT

Conference decision and notes (fillable)

Record the team's decision here. Saved with the feedback form.

Patient 9000345 | Episode 7000588 | Branch 0518 | Day 45 of 60

Conference decision

System discharge recommendation: Discharge (page 9)

☐ Agree with CQI ☐ Agree with changes ☐ Disagree ☐ Escalate to MD

Action owner	Due date	Reviewed by

Discussion notes

Follow-ups and owners

Anything else for the CQI team